

Recommendation

Time should be allowed to clearly explain the objectives to the patient, including therapeutic education, sun protection, and self-monitoring, at the time of diagnosis.

Initial Diagnostic Evaluation by Stage

Stages IA and IB

- Full clinical examination, particularly of the entire skin (integument) and lymph nodes.
- No systematic additional investigations are recommended.
(**Expert opinion**)

Stages IIA and IIB

- Full clinical examination, including complete skin and lymph node assessment.
- Ultrasound imaging of the regional lymphatic drainage basins.

While ultrasound has higher sensitivity than clinical examination alone for detecting lymph node metastases, no study has demonstrated a survival benefit from its use.

(**Expert opinion**)

Stages IIC and IIIA

- Full clinical examination of the skin and lymph nodes.
- Ultrasound imaging of the lymphatic drainage system (recommended for Stage IIC).

- Optional imaging with cerebral and thoraco-abdomino-pelvic CT (C-TAP CT) or ^{18}F -FDG-PET:
- Particularly considered if sentinel lymph node biopsy is planned (Stage IIIC) or prior to lymph node dissection (Stage IIIA).

Although ^{18}F -FDG-PET appears more sensitive than CT, no formal recommendation favoring one over the other can be made due to variable availability.

(Expert opinion)

Stages IIIB and IIIC

- Full clinical examination of the skin and lymph nodes.
- Imaging with ^{18}F -FDG-PET or C-TAP CT is recommended before surgery (e.g., lymph node dissection).

Imaging may detect nodal involvement beyond the standard dissection field and rule out synchronous distant metastases. Any suspicious distant lesions must be histologically confirmed when clinically indicated.

(Expert opinion)

Special Cases

- When considering adjuvant therapy (e.g., sentinel node biopsy or low-dose interferon), baseline imaging should be individually discussed to exclude more advanced disease.

(Expert opinion)

Immunohistochemistry Prognostic Markers in Melanoma

- There is no indication for the systematic use of immunohistochemical prognostic markers in stages I–III.
(Recommendation Grade A)

Role of Oncogenetic Consultation in Melanoma Management

An oncogenetic consultation is recommended in the following situations:

- At least two invasive melanomas in first- or second-degree relatives or in the same individual, diagnosed before age 75.
- Personal or family history of invasive cutaneous or ocular melanoma associated with:
 - Pancreatic cancer
 - Kidney cancer
 - Mesothelioma
 - Central nervous system tumor

Genes currently associated with melanoma risk include: - High-risk: *CDKN2A*, *CDK4*, *BAP1* - Intermediate susceptibility: *MC1R*, *MITF*

(Reference to French guidelines acknowledged; no additional literature review performed by this group)

Topic 5B: Follow-up

Stages IA and IB

- 5-year recurrence risk:
 - Stage IA: 1% to 7.1%
 - Stage IB: 1% to 18%

- Types of recurrence (in decreasing frequency):
- Local or in-transit cutaneous
- Lymphatic
- Distant
- Recurrences may occur after short or very long intervals — occasionally more than 10 years post-diagnosis.